

LYBALVI prescribing in Medicare Part D

United States | Service years 2023-2024 | Li Tian, PhD | October 2, 2026

Independent portfolio analysis of public CMS records. Not sponsored by or affiliated with Alkermes.

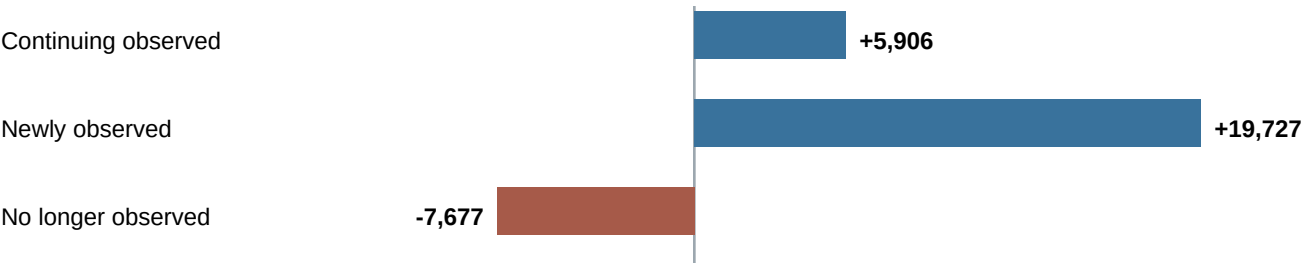
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Business question and main finding

How did observed LYBALVI prescribing change, and how much of the change came from prescribers appearing in both years? Published claims rose from **39,134 to 57,090**, an increase of **17,956 (+45.9%)**. Continuing observed prescribers contributed 5,906 additional claims; changes in which prescriber records appeared in the public files contributed a net 12,050.

Observed metric	2023	2024
LYBALVI claims	39,134	57,090
LYBALVI prescribers (distinct NPIs)	1,623	2,344
Claims across the four-product set	14,295,635	14,933,828
LYBALVI share within that set	0.274%	0.382%

Where the observed growth came from



The 1,132 prescribers observed in both years increased claims by 18.8%, from 31,457 to 37,363. Newly observed records contributed 19,727 claims and no-longer-observed records contributed -7,677. Contributions reconcile exactly to the 17,956-claim increase.

Interpretation for commercial analytics

The decomposition separates growth among consistently visible prescribers from changes in record visibility. A useful next investigation would compare specialty and geography within the continuing cohort, then test whether apparent changes persist across volume thresholds. These are descriptive follow-up questions, not sales targeting or ROI recommendations.

Critical caveat: CMS excludes provider-drug records with fewer than 11 claims. Newly observed does not mean newly prescribing; disappearance does not establish cessation. Claims include refills and are not unique patients.

Methods and analytical reliability

Scope and metric definitions

The analysis uses 558,954 published provider-drug-year records from 50 states and Washington, D.C. for 2023 and 2024. The selected comparison set contains exact CMS brand/generic matches for LYBALVI, Olanzapine, Aripiprazole, and Quetiapine Fumarate. Other brands are excluded; route and formulation are unresolved. This is neither the total market nor a set of clinically interchangeable treatments.

Selected-set share is observed LYBALVI claims divided by claims across those four products, calculated as a ratio of sums. It increased by approximately 0.108 percentage points. Prescriber counts are distinct NPIs within the specified year and product scope. Provider location is not patient residence; beneficiary counts are not summed.

Data workflow

Extract: Python retrieves paginated, filtered CMS API records using pinned release identifiers, request timeouts, retries, and cached source snapshots.

Transform: Python validates scope and identifiers; DuckDB executes reusable SQL for product, year, state, segment, and observed-growth reporting layers. NPI identifiers remain text and missing source values remain distinct from zero.

Validate and publish: Independent reconciliations check aggregates and cohort totals before staged outputs become the current validated snapshot. Eight aggregate CSVs support Tableau. Tableau uploads and republishing are manual.

Evidence of reliability

Two cached-input runs returned identical SHA-256 hashes for all nine exported CSVs (eight Tableau datasets and one detailed download). Deliberately injected duplicates, a missing required column, and negative claims were rejected without changing the current output pointer or valid CSV hashes. Verification completed on October 2, 2026; the repository includes a verification summary and selected aggregate evidence.

Limitations and responsible interpretation

Observed records reflect Medicare Part D public reporting, not all prescribing or all patients. Suppression limits interpretation of low-volume activity and cohort entry or exit. The data do not establish diagnosis, treatment suitability, marketing causality, manufacturer revenue, or sales potential. Annual historical data are not a live feed. Changes in prescriber attributes can also affect geographic comparisons.

Authorship and reproducibility

Li Tian authored the Tableau dashboard and reviewed the workflow and reconciliations. Codex assisted with code implementation, documentation, source inspection, and automated verification. Source code, configuration, requirements, metric definitions, and run instructions are included in the repository; raw downloads and the local Python environment are excluded. The first full rebuild requires CMS network access.

Sources and project links

[CMS Medicare Part D Prescribers by Provider and Drug](#) - 2023 and 2024 service-year releases.

Release IDs: 2023 e54db557-cd82-4e91-a0fe-61aad5865d69; 2024 d5aa71a8-dcc0-4570-8bcf-bd39deac69fe. Local analytical evidence: `year_summary.csv`, `growth_components.csv`, and `verification_summary.json`.

[Published Tableau dashboard](#) | [Li Tian on LinkedIn](#)